



OFFICIAL

NMHS MHS POLICY

Medication: Management of High-Risk Medications

Scope (Audience)	All NMHS Mental Health Services (MHS) Staff
Scope (Area)	All NMHS MHS Sites
Summary	Improve patient safety amongst staff regarding high-risk medications that have an increased risk of causing patient harm or death if misused or used in error. Giving information on safer systems for high risk and other medications.
Risk Rating	High
NSQHS Standards	Clinical Governance (Std 1) Medication Safety (Std 4)
Policy Sponsor	Director of Medical Services & Chief Pharmacist MHS
Policy Contact	Chief Pharmacist MHS
Version	4.0
Review Date	26/03/2027

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

High risk medications will be managed by NMHS MHS services according to evidence based practice in order to ensure patient safety and therapeutic outcomes.

2. Background

This policy identifies medications that are deemed high risk medicines and provides a standard for the management of high risk medications including storage, prescribing, dispensing and administration.

This document reflects current safe practice standards. However, as in any clinical situation, there may be factors which cannot be covered by a single set of guidelines. It does not replace the need for the application of clinical judgement to each individual presentation.

This policy is to be read in conjunction with:

- [WA High Risk Medication Management Policy MP 0131/20](#)
- [Medicines Handling Policy MP 0139/20](#)
- [Clinical Incident Management Policy MP 0122/19](#)
- [National Safety and Quality Health Service Standards: Medication Safety](#)

The Australian Commission for Safety and Quality in Health Care (ACSQHC) recommends the following medications represented with the acronym, APINCHS must be included in a high risk medication list.

The list is based on the WA High Risk Medication Management Policy APINCHS list:

A Antimicrobials

P Potassium and other electrolytes; Psychotropic medications

I Insulin

N Narcotics/ Opioids; Neuromuscular blocking agents

C Chemotherapeutic agents

H Heparin and other anticoagulants

S Safer Systems (e.g. safe administration of liquid medications; independent double checks, medication charts; clinical handovers).

3. Risk

Patients may be at increased risk of experiencing adverse events including significant or catastrophic harm or death when high risk medicines are misused or used in error.

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐

Medical Equipment	☐	Other	☐
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4. Policy Principles

In line with the National Safety and Quality Healthcare Service (NSQHS) Medication Safety Standard, NMHS MH needs to:

- Identify high risk medications used within the organisation
- Have systems to store, prescribe, dispense and administer high risk medications safely.

The High Risk Medication Register will be determined and maintained by the NMHS MHS Drugs & Therapeutics Committee (DTC).

Clinical staff need to be aware of the medication listed in the register, and the need to take extra care in the safe storage, handling, prescribing, dispensing, administration, disposal and monitoring of high risk medication.

Errors with high risk medication may not necessarily be more common, but the consequences of these errors may be more devastating.

The register will be reviewed annually and will be revised as new safety information or new medications are added.

Before a new medicine is placed on the WA Statewide Formulary ([Formulary One](#)), a safety risk assessment will be performed by the WA Drug Evaluation Panel (WADEP). Where a medicine is identified as being a high risk medication, the medicine will be included in the NMHS MHS High Risk Medication Register and appropriate policies and protocols will be developed.

5. Clinical Incident Management

Any clinical incidents involving high risk medications (including near misses) must be reported as per the [WA Clinical Incident Management Policy](#).

6. Safer Systems

- Using non-Luer lock oral syringes for oral, enteral or nebuliser liquid administration
 - [NMHS MH Medication: Safe Administration of Liquid Medications](#)
- Standardised abbreviations when writing medication charts
 - [Recommendations for safe use of medicines terminology](#)
 - [Recommendations for safe use of medicines terminology - fact sheet](#)
- Using standardised medication charts:
 - WA Hospital Medication Chart (WA HMC); WA Adult Clozapine Initiation and Titration Chart; Agitation and Arousal PRN Chart; Intramuscular Long-Acting Injection Chart (Depot Antipsychotics); WA Adult Anticoagulation Chart; Insulin Chart.
 - [WA Medication Charts](#)
- Use of Decision Support Tools such as [Don't Rush to Crush+](#) before administering medications.
- Clinical Handovers, using the iSoBAR tool, must identify high risk medications prescribed for patients, including treatment plans.

i	Identify	Identify high risk medication
S	Situation	Clinical situation: reason patient is on this medication; risks associated
o	Observations	Observation/ strategies for mitigating harm; monitoring
B	Background	Background for patient; concerns
A	Agreed Plan/ Assessment	Specific assessment criteria; blood tests; monitoring
R	Readback/ risk/ recommendation	Review medication charts; document any issues, monitoring, changes

7. High Risk Medication Register

The following medications have been identified by NMHS MHS Drug & Therapeutics Committee as high-risk medications. The relevant NMHS MHS clinical policies and/or guidelines are included. [WA High Risk Medication Policy MP 0131/20](#).

This is not an exhaustive list of high risk medications, all medications carry risk of adverse events if prescribed, administered or dispensed inappropriately.

MEDICATION	POLICIES/ CLINICAL GUIDELINES	COMMENTS
Antimicrobials	NMHS MH Antimicrobial Prescribing and Stewardship Policy Formulary One Therapeutic Guidelines: Antibiotic ACSQHC Antimicrobial Stewardship Clinical Care Standard	Prescribing outside these guidelines requires the approval of a clinical microbiologist.
Psychotropic Medications	Clozapine NMHS MH Clozapine Prescribing Policy Clozapine Initiation and Titration Chart WA Mental Health Charts and Clozapine Resources Guidelines on the Safe and Quality Use of Clozapine in the WA health system Clopine Central®	Strict monitoring is required due to its potential to cause neutropenia, agranulocytosis, myocarditis, cardiomyopathy and severe constipation (Clozapine Induced Gastrointestinal Hypomotility (CIGH)).
	Zuclopenthixol Acetate	Restricted on Formulary One: Acute arousal and agitation as part of the Hospital's acute

	NMHS MH Zuclopenthixol Acetate (Clopixol Acuphase®) Prescribing Policy	agitation and arousal guideline
	Long-Acting Antipsychotics Injections NMHS MH Medication: Intramuscular Administration of Long-Acting Antipsychotics	
	Lithium NMHS MH Lithium: Prescribing & Management	Narrow margin between therapeutic & toxic dosages. Monitoring required to avoid lithium toxicity, including levels, renal function, urea & electrolytes
	Psychotropic Medicines in Cognitive Disability or Impairment Clinical Care Standard Australian Commission on Safety and Quality in Health Care NMHS MH Medication: Off-Label Prescribing Including Prescribing Greater than Maximum Dose	
Insulin	NMHS MH Insulin Prescribing and Administration WATAG Safety Alert: High Concentration Insulin WAMSG High concentration insulin - Toujeo® ACSQHC Discontinuation of multiple insulin products FSFH Poster: Know your insulins	Prescribe insulin by brand/trade name, specify device. Order as 'UNITS' not 'U' or 'IU'. Specify time of administration. Label and store correctly.
Narcotic Transdermal patches (TDP)	NMHS MH Medication: Transdermal Patch Management	Confirmation of TDP placement is best practise when the patch is left in situ for more than 24hours

Narcotic/ Opioids & Sedative Agents including: Methadone Buprenorphine	WA Monitored Medicines Prescribing Code WA Pain relief medications following surgery or injury WA Recommendations for prescribing analgesia on discharge following surgery or acute injury ACSQHC Opioid Analgesic Stewardship in Acute Pain Clinical Care Standard WA Guideline on continuation of opioid substitution treatment in hospitals WA Community Program for Opioid Pharmacotherapy (CPOP) WA Guidelines for use of depot buprenorphine (Buvidal® and Sublocade®) in the treatment of opioid dependence Opioid Pharmacotherapy in Hospitals: Continuation, Withdrawal, Initiation and Discharge Arrangements Guideline	Caution required with elderly patients & patients with impaired clearance. 'Start low & go slow' Regular review of analgesia; monitoring sedation and respiratory levels. Outlines the administration of this treatment when patients admitted to hospital.
Neuromuscular Blocking Agents	SCGOPHCG High Risk Medications Policy ACSQHC National standard for user-applied labelling of injectable medicines, fluids and lines	SCGH MHU Day Procedure Unit (DPU) stocks on imprest: • Suxamethonium • Rocuronium Stored in a lock fridge on DPU.
Chemotherapeutic Agents (cytotoxic)	NMHS MH Medication: Management of Hazardous Medicines	Patients & workers handling or coming into contact with hazardous medicines may be exposed to the risk of adverse effects which can be harmful.
Heparin, warfarin & Direct Oral Anticoagulants (DOACs) (also referred to as NOACs)	Guidelines for the WA Anticoagulation Medication Chart (WA AMC) Living with Warfarin Clinical Excellence Commission NOAC Guidelines for Non-Vitamin K Antagonists Oral Anticoagulants	Warfarin has potential to interact with many medications. There are 2 brands (Marevan®, Coumadin®) that are not bioequivalent. DOACs (or NOACs): apixaban, dabigatran, rivaroxaban.

	Living with a direct-acting oral anticoagulant (DOAC) WA Poster: Know your DOACs aka NOACs	Low Molecular Weight Heparins & Heparinoid: enoxaparin, danaparoid, dalteparin, nadroparin
Intravenous Potassium (IV) Vinca Alkaloids	These classes of medications are not to be used at NMHS MHS sites. If a patient requires these medicines, they must be referred to a service with the capabilities to administer these classes of medications.	

8. Roles and Responsibilities

Roles	Responsibilities
NMHS MH Drug and Therapeutic Committee (DTC)	Is responsible for: • Determining and maintaining the high-risk register. • Reviewing reports on high-risk medication incidents. • Review and endorsing policies, procedures and guidelines specific to high-risk medications.
Clinical staff involved in medication management	Are responsible for: • Following requirements and recommendations outlined within this document and the related policies, procedures and guidelines. • Ensuring close monitoring of patients receiving high-risk medications and comprehensive patient education provided. • Managing clinical incidents involving high-risk medications. • Maintaining up to date knowledge and best practices for the safe handling and use of high risk medications. • Working within their scope of practice and apply sound clinical judgement to patient care.
Prescribers	Are responsible for: • Prescribing high-risk medications clearly and safely using the endorsed medication chart or prescriptions. • Ensure ongoing review and monitoring of these medications to support safe and effective patient care.
Pharmacists	Are responsible for: • Providing clinical review of patients on high-risk medications • Ensuring safe storage, supply and handling of high-risk medications.
Nurses	Are responsible for: • Safe storage, administration, monitoring and handling of high-risk medications.

All NMHS staff	Are responsible for: • Having knowledge of the policy documents relevant to their roles and how to access these documents. • Complying with mandatory requirements outlined in these documents. • Assisting with policy development/reviews wherever required. • Reporting to their immediate line manager any potential non-compliance or implementation issues. • Completion of NMHS High Risk Medicines Education Learning Path is highly recommended
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9. Monitoring and Evaluation

Compliance with this policy is mandatory. There are legislative obligations relating to this policy where a statutory penalty may apply for non-compliance.

Use of high-risk medications will be monitored by MHS Pharmacy & reported to MH DTC.

- Datix CIMS reports on medication incidents will include reference to high-risk medications.
- Periodical audits will be done against individual NMHS MH & NMHS Medicines policy.

WA Health Compliance Audit Tool for monitoring Health Service Providers compliance to the mandatory requirements of the [MP 0131/20 High Risk Medication Management Policy](#). [High Risk Medication Management Policy Compliance Audit](#) is to be submitted annually by 28th February. A [fillable PDF Document](#) of the audit tool is provided as a visual reference, or for internal data collection only.

This policy will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed annually using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

10. Definitions

Terms	Definitions
High Risk Medications	Medications that have a heightened risk of causing significant or catastrophic harm when used, either as prescribed or in error. This includes medications with a low therapeutic index and medications that present a high risk when administered via the wrong route or when other systems errors occur.
High Risk Register	A register consisting of a list of medications or medication groups considered to be 'high-risk'.

11. Related Documents

Legislation	• WA Medicines and Poisons Act 2014 • WA Medicines and Poisons Regulations 2016
Government policies	• WA High Risk Medication Management Policy MP 0131/20 • Medicines Handling Policy MP 0139/20 • Clinical Incident Management Policy MP 0122/19

WA Health mandatory policies	• WA Medication Chart Policy MP 0078/18
NMHS MH Policy documents	• NMHS MH Lithium: Prescribing & Management • NMHS MH Insulin Prescribing and Administration • NMHS MH Antimicrobial Prescribing and Stewardship Policy • NMHS MH Clozapine Prescribing Policy • NMHS MH Zuclopenthixol Acetate (Clopixol Acuphase®) Prescribing Policy • NMHS MH Medication: Management of Hazardous Medicines • NMHS MH Medication: Transdermal Patch Management
Standards	• NSQHS Standards

12. Document Control

Document Version Control			
Date	Version	Author	Notes
14/5/2015	V1.0	MHEG	Email out of session
10/06/2015	V1.0	A/Executive Director (P. Marwick)	
11/10/2018	V2.0	MHEG	Approved out of session – email
19/10/2018	V2.0	Executive Director	Endorsed via Submission Form
19/10/2018	V2.0	NMHS Chief Pharmacist	Scheduled review: change of policy title (previously High Risk Policy), updated links to local and external policies
18/07/2019	V2.1	A/Policy & Audit Officer	Updated hyperlinks. Removed reference to NMHS MH Vancomycin Policy (rescinded) No change to scheduled review dates. Updated the NSQHS Standards V2.0.
27/08/2020	V2.2	NMHS MH Chief Pharmacist	Links updated. New WA High Risk Medication Policy linked, no change to scheduled review date.
23/05/2022	V3.0	NMHS MH Chief Pharmacist	Scheduled review; new template; updated hyperlinks; new information & links added. High risk register updated.
26/03/2026	V4.0	NMHS MH Chief Pharmacist	Removal of and update information regarding the WA High Risk Medication Management Policy. Updated the hyperlinks. High Risk Register information updated; removal of information no longer available. Addition of statement regarding IV potassium and Vinca Alkaloids.

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community. ISD Number: 5442

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